

Medication Administration

Procedures for Safe Drug Dispensing and Tracking

Regulation: 42 CFR §482.25

POLICY OVERVIEW

Medication safety is one of the highest-priority patient safety concerns in hospitals. Medication administration policies must address the entire medication use process from ordering through administration and monitoring, with robust safeguards at each step.

KEY REQUIREMENTS

1 The Five Rights (Minimum Standard)

Before administering any medication, staff must verify: Right Patient (two patient identifiers), Right Drug, Right Dose, Right Route, and Right Time. Many facilities have expanded to seven or ten rights.

2 Medication Reconciliation

A complete and accurate medication list must be obtained at admission, reconciled at each transition of care, and provided to the patient/caregiver at discharge. Discrepancies must be resolved by a licensed provider.

3 High-Alert Medications

Medications with heightened risk of harm (anticoagulants, insulin, concentrated electrolytes, opioids, chemotherapy) must have additional safeguards including independent double-checks, special labeling, and restricted access.

4 Controlled Substance Management

Controlled substances must be stored in double-locked secure areas, counted at each shift change, and all waste must be witnessed and documented. Discrepancies must be reported and investigated immediately.

5 Adverse Drug Event Reporting

All adverse drug events, near-misses, and medication errors must be reported through the incident reporting system and analyzed for root cause to prevent recurrence.

COMPLIANCE NOTES

The Pharmacy and Therapeutics (P&T;) Committee oversees medication management policies. A pharmacist must review medication orders before administration except in urgent situations. Pharmacist participation in patient care rounds is recommended.

COMMON SURVEY CITATIONS

Frequently cited under §482.25(b) – Pharmaceutical services policies, and §482.25(b)(3) – Drug storage and security.